



The Future of 340B: Trends and Predictions

A Strategic Outlook for Pharmacy Executives and 340B Program Administrators

Executive Summary

*The 340B Drug Pricing Program stands at a critical inflection point in 2025, with transformative regulatory changes, unprecedented market consolidation, and revolutionary technology adoption reshaping its landscape. With **\$66.3 billion** in outpatient drug purchases in 2023 and nearly **42,000 covered entities** serving over 53,000 care sites, the program's strategic importance has never been greater.*

Key findings reveal that covered entities are achieving significantly improved compliance rates (62% decrease in audit findings), while manufacturer restrictions continue to expand with 39 companies now limiting contract pharmacy access. The introduction of HRSA's Rebate Model Pilot Program in August 2025, coupled with proposed oversight transition from HRSA to CMS, signals fundamental structural changes ahead.

1. Current State of the 340B Drug Pricing Program (2025)

Program Scale and Scope

The 340B Program has reached unprecedented scale in 2025, encompassing nearly **42,000 covered entities** operating across more than **53,000 care sites**. This represents a dramatic expansion from the program's modest beginnings in 1992 with just 1,000 covered entities and registered sites. The program's financial impact is equally substantial, with covered entities purchasing **\$66.3 billion** in outpatient drugs under 340B pricing in 2023.

Covered Entity Composition

The current landscape is dominated by hospital participation, with more than **40% of all hospitals** in the United States participating in the program. Critical Access Hospitals (CAHs) and Disproportionate Share Hospitals (DSHs) represent the largest segments, accounting for 38% and 32% respectively of audited covered entities in FY 2024.

Contract Pharmacy Growth

The contract pharmacy network has experienced explosive growth, expanding from roughly **1,000 pharmacies in 2010** to over **25,000 by 2022**, with current data showing approximately **32,000 unique contract pharmacy locations**. This represents nearly 60% of the entire U.S. pharmacy industry.

Market Consolidation

The market is characterized by extreme consolidation, with the top five companies—CVS Health, Walgreens, UnitedHealth Group (Optum Rx), Cigna (Express Scripts), and Walmart—controlling **76.1% of all contractual relationships**, up from 55.9% in 2016.

2. Recent Regulatory Changes and Proposed Legislation

HRSA Rebate Model Pilot Program

The most significant regulatory development of 2025 is HRSA's launch of the **340B Rebate Model Pilot Program** in August. This voluntary program, effective January 1, 2026, represents a fundamental shift in how 340B discounts may be provided, moving from upfront pricing discounts to retrospective rebate mechanisms.

Key Program Features

- Limited to manufacturers with Medicare Drug Price Negotiation Program (MDPNP) agreements
- Applies only to NDC-11s included on the MDPNP drug list
- Manufacturers bear all IT platform costs
- 60-day implementation notice requirement
- 10-day rebate payment timeline
- Minimum one-year participation commitment

Proposed Oversight Transition

The U.S. Department of Health and Human Services' proposed budget for fiscal year 2026 includes a significant structural change: **transferring 340B program oversight from HRSA to the Centers for Medicare and Medicaid Services (CMS)**. This transition would provide enhanced regulatory authority and enforcement capabilities, addressing long-standing criticism about HRSA's limited rulemaking power.

Medicare Part B Billing Requirements

CMS implemented mandatory billing modifier changes effective January 1, 2025, requiring all 340B covered entities to transition to the **"TB" modifier** for separately payable Medicare Part B drugs and biologics. This change from the previous "JG" modifier system

enhances tracking capabilities and supports duplicate discount prevention efforts.

State Legislative Developments

Eight states have enacted legislation prohibiting manufacturers from restricting 340B-priced drugs to contract pharmacies in 2025: Arkansas, Kansas, Louisiana, Maryland, Minnesota, Mississippi, Missouri, and West Virginia. An additional two states (Michigan and Ohio) have pending legislation, indicating a growing state-level response to manufacturer restrictions.

3. Key Trends Affecting 340B Programs

Market Consolidation Acceleration

The most significant trend affecting 340B programs is the accelerating consolidation across all market segments. The contract pharmacy market demonstrates this clearly, with the top five companies increasing their market share from 55.9% in 2016 to 76.1% in 2025.

Manufacturer Restriction Expansion

Thirty-nine drug manufacturers now impose limitations on 340B contract pharmacy arrangements, representing a substantial increase from previous years. These restrictions take various forms, including:

- Direct dispensing requirements
- Limited pharmacy network access
- Rebate model implementations
- Enhanced data submission requirements

Compliance Improvement Success Story

Covered entities have demonstrated remarkable improvement in compliance metrics. Analysis of federal audit data shows a **62% decrease** in combined diversion and duplicate discount findings among 340B hospitals.

- 57% decrease in duplicate discount findings (from 30.8% to 13.2%)
- 73% decrease in diversion findings (from 39.7% to 10.7%)
- 61% decrease in findings requiring repayment (from 71% to 28%)

4. Technology and Data Analytics Impact

Artificial Intelligence Integration

AI technology is revolutionizing 340B program management through several key applications:

Automated Data Processing

- Real-time ingestion of data from multiple formats (Excel, CSV, PDF, JSON, HL7v2)
- Automatic conversion of disparate data into standardized formats
- 360-degree view across multiple contract pharmacies and eligible locations

Machine Learning Applications

- Savings opportunity identification through configurable rule-based logic
- Pattern recognition for compliance risk assessment
- Predictive analytics for audit preparation

Software Solution Evolution

The 340B technology vendor landscape has evolved significantly, with major players including:

- **Craneware Group:** Revenue integrity and 340B management with integrated data processing
- **McKesson Macro Helix:** Comprehensive 340B drug program software and technology solutions
- **Bluesight 340BCheck:** Compliance software designed for continuous monitoring
- **Plenful:** AI-powered platform for auditing optimization and savings identification

5. Contract Pharmacy Relationships Evolution

Market Structure Transformation

The contract pharmacy landscape has undergone fundamental structural changes, evolving from a distributed network of independent pharmacies to a highly concentrated market dominated by major chains and PBMs.

Metric	Current State (2025)	Growth/Change
Unique Contract Pharmacy Locations	32,000	Nearly 60% of U.S. pharmacy industry
Total Contractual Relationships	229,531	21% CAGR from 2016-2025
Top 5 Companies Market Share	76.1%	Up from 55.9% in 2016
Independent Pharmacy Share	15%	Declining due to resource constraints

Specialty Pharmacy Integration

The integration of specialty pharmacies into 340B networks has accelerated, with PBM-affiliated specialty pharmacies experiencing rapid growth. Non-retail relationships (mail, specialty, infusion) for the three largest PBMs increased from **14,000 in 2020 to 57,000 in 2025**.

6. Compliance and Audit Considerations

HRSA Audit Landscape

The FY 2024 audit results provide critical insights into current compliance expectations:

FY 2024 Audit Statistics

- **111 audit results** posted as of December 2024

- **54% of audits** resulted in no adverse findings
- Coverage across **40 states**
- **81% of audits** focused on hospitals (CAHs and DSHs dominating)

Common Compliance Findings

The most frequent audit findings in FY 2024 reveal persistent compliance challenges:

1. "Incorrect 340B OPAIS Record" (62% of findings):

- Incorrect Medicare Cost Report filing dates
- Mismatched entity information
- Improper contract pharmacy registrations

2. "Duplicate Discount" (17% of findings):

- Inaccurate or incomplete Medicaid Exclusion File (MEF)
- Billing system errors
- Claims processing deficiencies

3. "Diversion" (17% of findings):

- Contract pharmacy prescriptions from unregistered locations
- Ineligible patient dispensing
- Documentation deficiencies

7. Strategic Predictions for the Next 3-5 Years (2025-2030)

Regulatory Transformation

Oversight Transition (2026)

The proposed transfer of 340B oversight from HRSA to CMS will likely occur, bringing enhanced regulatory authority and more frequent rule-making. This transition will enable:

- More prescriptive compliance requirements
- Enhanced enforcement capabilities
- Improved coordination with Medicare/Medicaid programs
- Standardized audit procedures

Rebate Model Expansion (2026-2028)

The current pilot program will likely expand beyond MDPNP drugs, potentially becoming a standard option for all manufacturers. This evolution will require:

- Enhanced IT infrastructure investments
- Modified financial forecasting models
- Revised contract pharmacy agreements
- Updated compliance procedures

Market Consolidation Continuation

Contract Pharmacy Concentration (2025-2030)

Market concentration will continue, with the top five companies potentially controlling **85%+** of contractual relationships by 2030. This will result in:

- Reduced negotiating power for smaller covered entities

- Standardized service offerings across major chains
- Enhanced technology integration requirements
- Potential pricing standardization across networks

Health System Integration

Hospital systems will increasingly acquire or partner with specialty pharmacies to maintain control over 340B revenue streams, leading to:

- Vertical integration acceleration
- Reduced reliance on external contract pharmacies
- Enhanced data control and compliance management
- Improved patient outcome tracking

Technology Requirements Evolution

Mandatory Automation (2026-2028)

Advanced technology solutions will become essential rather than optional, driven by:

- Increasing compliance complexity
- Manufacturer data submission requirements
- Real-time audit preparation needs
- Regulatory reporting enhancements

AI Integration Standardization (2027-2030)

Artificial intelligence will become standard across 340B programs, enabling:

- Predictive compliance monitoring
- Automated savings optimization
- Real-time error detection and correction
- Advanced analytics for strategic planning

8. Actionable Recommendations for 340B Covered Entities

Short-Term Tactical Recommendations (2025-2026)

1. Immediate Compliance Actions

- Implement TB modifier requirements for Medicare Part B billing effective January 1, 2025
- Conduct comprehensive OPAIS audit to ensure accuracy across all registered sites
- Review and update Medicaid Exclusion File procedures for enhanced accuracy
- Establish contract pharmacy registration verification protocols

2. Technology Infrastructure Assessment

- Evaluate current 340B management systems for AI and automation capabilities
- Assess data integration capabilities across pharmacy, EMR, and billing systems
- Develop vendor evaluation criteria for potential technology upgrades
- Create budget allocations for mandatory technology investments

3. Rebate Model Preparation

- Analyze current drug portfolio for MDPNP inclusion
- Develop financial models for rebate timing impact
- Establish manufacturer communication protocols
- Create IT platform integration procedures

Medium-Term Strategic Recommendations (2026-2028)

1. Advanced Technology Implementation

- Deploy AI-powered 340B management platforms with predictive analytics capabilities
- Implement real-time compliance monitoring systems
- Establish automated audit preparation procedures
- Integrate advanced data analytics for savings optimization

2. Contract Pharmacy Strategy Refinement

- Evaluate contract pharmacy relationships for performance and compliance
- Negotiate enhanced data sharing agreements with key partners
- Consider vertical integration opportunities for specialty pharmacies
- Develop alternative partnership models for independent pharmacies

Long-Term Strategic Recommendations (2028-2030)

1. Market Position Optimization

- Develop comprehensive 340B value proposition for payer negotiations
- Create patient outcome measurement and reporting capabilities
- Establish 340B program integration with population health initiatives
- Build strategic partnerships with technology vendors and service providers

2. Organizational Capability Building

- Establish dedicated 340B centers of excellence
- Develop specialized compliance and technology expertise
- Create cross-functional 340B leadership teams
- Implement continuous improvement processes for program optimization

Conclusion

The 340B Drug Pricing Program stands at a transformative moment, with regulatory changes, market consolidation, and technology evolution creating both challenges and opportunities for covered entities. Organizations that proactively adapt to these changes while investing in advanced technology solutions and compliance capabilities will emerge as leaders in the evolving 340B ecosystem.

Success in the future 340B landscape will require a combination of strategic vision, operational excellence, and technological sophistication. Covered entities must begin preparing now for the changes ahead, ensuring they have the capabilities and partnerships necessary to thrive in an increasingly complex and regulated environment.

The recommendations outlined in this analysis provide a roadmap for navigating the next five years of 340B program evolution. By implementing these strategies systematically and maintaining a focus on compliance, technology adoption, and strategic positioning, covered entities can maximize the value of their 340B programs while contributing to the program's mission of supporting safety net healthcare providers.